



Restore

AI that wins dental claims back.

Pre-authorizations, claim appeals, and referral letters — drafted from your PMS, approved in one click. Carrier intelligence that gets smarter every week.

restore-demo.vercel.app · github.com/rrmethodco/rrmethodco

Carriers deny 20-30% of valid claims. Most practices give up.

An office manager bills at ~\$40/hr. A 90-minute appeal letter for a claim that *might* get paid in three months is a losing bet — so it doesn't get written.

Endo specifically: \$5K-\$15K claims routinely written off because no office manager has 90 minutes to fight a carrier.

And generic AI text generators don't fix this. Every carrier wants a different narrative. Without per-carrier intelligence, generic appeals lose 60-70% of the time.

THE MATH TODAY

\$500K–\$2M

annually written off per practice in justified revenue

90+ min

of office-manager time per appeal — labor cost often exceeds expected recovery

35-50%

success rate when practices draft appeals themselves

No learning loop

every appeal a practice writes loses its lessons the moment the case closes

Why practices don't appeal — even when they should.

Time cost > revenue probability

Office manager billable at ~\$40/hr. 90 min appeal = \$60 cost. 50% win × \$500 avg = \$250 EV. The practice breaks even at best.

Every carrier wants a different narrative

Cigna wants anatomical specificity. Delta wants ADA-CDT-exact terminology. MetLife wants conservative-alternatives-considered language. Generic letters lose 60-70% of the time.

No learning loop

This appeal lost? Next case gets the same flawed template. Practice has zero visibility into why denials cluster. Carrier intelligence dies with the person who drafted the letter.

Three converging shifts make this practical in 2026.

01

LLM reasoning extracts carrier patterns at scale

Read 1,000 EOBs across one carrier, identify denial clusters. Inference cost <\$0.10 per narrative. Vision models parse claim forms and clinical notes at speed.

02

Dental PMS integrations have matured

PBS Endo, TDO, Endovision, Dentrix, Open Dental all stream events in real time. New TX plans, EOBs, completed treatments — live webhooks. No new login friction for staff.

03

Sophisticated buyers + BAA-grade AI

DSO consolidation (16% → 32% of practices) creates RCM-aware buyers. Endo measures insurance leakage as a P&L line. Anthropic API ships BAA out of the box.

Watch. Draft. Approve. Done.

Restore lives inside the practice's PMS. When something happens that needs a response, we draft it. The office manager reviews and submits in one click. Submission to clearinghouse + PMS document center is autonomous.

01

Trigger

PMS webhook fires. New TX plan, EOB with denial code, treatment complete.

02

Draft

Restore reads case context. Per-carrier intelligence applied. Confidence rationale shown. Sometimes the answer is DO NOT APPEAL.

03

Review

Office manager opens inbox. 60-90 second review. Approve, edit, or skip with reason.

04

Submit

Auto-submits to clearinghouse. Logs to PMS Doc Center. Audit trail captured.

Result: 90 minutes of work compressed to 90 seconds of review.

Carrier intelligence that compounds.

Every approved claim makes the next narrative smarter. Every denied claim teaches us not to make that mistake again — across the customer base.

Denial patterns

Cigna: 75% of denials cluster on codes X, Y, Z. Delta: 41% on D3331 bundling. MetLife: 36% on apicoectomy 'try retreatment first.'

Narrative preferences

Cigna: lead with anatomical specificity. Delta: ADA-CDT-exact terminology. Aetna: front-load financial impact.

DO NOT APPEAL patterns

Plan-level exclusions, exhausted caps, contractual limits. Appealing degrades carrier credibility for the cases that DO matter.

Escalation paths

When peer-to-peer. When DOI complaint. Which carriers respond on first ask vs. require 3 attempts.

1 practice finds a pattern → 1,000 practices learn instantly. The intelligence isn't the AI; it's the feedback loop.

Three things no one else pairs together.

Capability	Restore	Vyne Dental	DentalRobot	Pearl AI
Carrier intelligence (per-carrier)	✓	Legacy rules	Generic	—
Case lifecycle threading	✓	Query-based	Single doc	—
DO NOT APPEAL discipline	✓	—	—	—
Real-time PMS integration	✓	Manual upload	Manual upload	Imaging only
Autonomous submission	✓	Draft only	Draft only	—

The gap we fill: carrier intelligence that learns + case threading that doesn't break + the discipline to say "don't appeal."

\$135M SOM. \$870M SAM. \$4.5B+ TAM.

Bottom-up. Endo first because density of pre-auth + appeal events per practice is 3-5× general dentistry.

SOM

Endodontic practices

\$135M

4,486 practices · \$30K ACV

PBS Endo + TDO + Endovision coverage. Win endo, the carrier intelligence layer compounds.

SAM

Dental specialty (endo, OS, perio, ortho, pedo)

\$870M

29,015 practices · \$30K ACV

All ADA-recognized specialties. Insurance-heavy workflows.

TAM

All US dental + DSO premium

\$4.5B+

178,000+ practices · \$25K blended

GP at lower ACV. DSO contracts (\$50-150K build + \$5-15K/mo) lift blended ARPU.

14-day customer payback. 86% gross margin.

CUSTOMER ROI

4-doc endo practice

Subscription	\$18,000/yr
Revenue recovered	\$214,000/yr
Hours saved (725 × \$35)	\$25,375/yr
Total annual value	\$239,000
Payback period	14 days
ROI multiple	13.3×

SAAS ECONOMICS

Per practice

ACV (sub + amortized build)	\$22,167
Gross margin	86%
CAC (Year 1 blended)	\$9,500
CAC payback	5.0 months
Logo churn (assumption)	5% annual
Net dollar retention	115%
LTV (4yr life)	\$74,500
LTV / CAC	7.8×

\$0 → \$5.4M ARR in 36 months.

Period	Practices	MRR (\$K)	ARR (\$K)	GP (\$K)	Opex (\$K)
Q4'26 pilot	1	\$1.5	\$18	\$15	\$180
Q2'27	8	\$13	\$156	\$134	\$310
Q4'27 — break-even	28	\$50	\$600	\$516	\$475
Q2'28	65	\$128	\$1,536	\$1,321	\$640
Q4'28	130	\$260	\$3,120	\$2,683	\$880
Q4'29	240 + 4 DSOs	\$450	\$5,400	\$4,644	\$1,400

ASSUMPTIONS

Logo churn 5% · NDR 115% (skill expansion + DSO upsell) · CAC \$11K → \$7.5K Y1→Y3 · Build fees deferred over 12mo · Cash break-even Q1'29 at ~\$3M ARR · 4 DSO contracts in Y3 = \$360K incremental ARR

Three tiers. Same engine. Different leverage.

We don't price per seat. We price per insurance complexity.

Solo Practice

1-2 doc endo / specialty

Build: \$10,000

Subscription: \$1,250/mo

→ **\$120K+ recovered/yr**

- All 3 core skills
- 1 PMS integration
- Standard support

Group Practice

3-8 doc, multi-loc

Build: \$15,000

Subscription: \$2,000/mo

→ **\$250-500K recovered/yr**

- All Solo features
- Multi-location dashboard
- Carrier benchmarking
- Priority queue

DSO / Enterprise

10+ practices

Build: \$50K-\$150K

Subscription: \$5K-\$15K/mo

→ **\$2M+ recovered/yr at 50 locations**

- All Group features
- SSO + custom BAA
- Custom carrier playbooks
- API access
- Dedicated CSM

From pilot to \$5M ARR in three channels.

Channel	Y1-Y3 logos	CAC	Conversion	Logic
AAE Annual + regional	60	\$8K	8% booth→close	Endo's mecca · 8K members we exhibit Y1, sponsor Y2
Study clubs / KOL referrals	110	\$4K	25% warm→close	Endo is a small world · people becomes 4 case studies · feedback loop
Cold outbound (top-200 endo groups)	50	\$14K	6% BDR→close	Apollo + ZoomInfo list · founder-led · high-ACV
DSO BD (Heartland, Pacific, MB2)	4 DSOs / 20 logos	\$35K	1 of 8 convs	Long cycle (9-12mo) · founder-led · PE-friendly economics

Blended CAC \$8.5K vs. ACV \$22.2K = 2.6× first-year payback. 240 endo practices = 5.3% of addressable endo market.

Funnel example (study club): 200 referred convos → 80 demos → 40 pilots → 28 paid (35% pilot-to-paid)

From pilot to playbook to portfolio.

PILOT June 2026 → December 2026

- Design partner: founder's mother's 4-doc endo practice (PBS Endo)
- Live: pre-auth narratives, claim appeals (with DO NOT APPEAL scoring), referrals
- Baseline appeal success: 40% → Target: 60%+ by month 4
- Expected outcome: \$150-200K annual revenue impact (1 practice)

YEAR 1 Jan – Dec 2027

- 50 endo practices live
- 10,000+ claims/mo flowing through carrier intelligence layer
- PBS Endo + TDO + Endovision integrations hardened
- Cohort avg appeal success rate: 55%

YEAR 2+ 2028 onward

- Specialty expansion: oral surgery (750), perio (1,200), ortho (2,500)
- DSO platform integrations: Heartland, Pacific Dental, MB2
- Network effects on carrier intelligence compound
- Long-term: carriers themselves licensing Restore for adjudication

Restore as infrastructure.

In five years, Restore is the operating system for dental insurance — inside practices and inside carrier networks.

Y1-Y2

Practice insurance operations

Restore = the brain for appeal strategy + pre-auth management. Endo → Oral surgery → Perio → Ortho → Pedo → GP.

Y3-Y4

DSO intelligence layer

Heartland / Pacific / MB2 license Restore for portfolio intelligence. DSOs use it for portfolio-level insurance revenue optimization.

Y5+

Carrier adjudication (the inverse market)

Carriers license Restore to grade appeals. "Should we approve or deny this?" Restore becomes the industry standard.

Same intelligence, compounding value at every layer. \$150B dental market with \$40B insurance friction.

Hospitality operator. Honest design partner. Financial discipline.

FOUNDER

Ross Richardson

- EVP Finance & Accounting, Method Co. (Philadelphia hospitality, 10 hotels + F&B)
- Led financial operations across 2,000+ employees
- Obsessed with: systems that scale, unit economics that don't lie, operator feedback loops
- Zero prior healthcare startup experience — zero dogma

DESIGN PARTNER

Allyson A. Abbott DMD PC

- 15+ years operating • multi-doc endodontic practice on PBS Endo
- Weekly product feedback (she tells me what doesn't work in 30 sec)
- Not a cheerleader — a customer who will switch if it's broken
- Product-market fit pressure-tests in real time

ADVISORY THESIS

Building

- Insurance ops experts (RCM, appeal specialists)
- Dental PMS integration partners (PBS Endo, TDO, Dentrrix)
- DSO executives (Heartland, Pacific, MB2)
- Carrier contacts for early intelligence loop

Founder who ships • customer who keeps honest • unit-economics flywheel from day one.

\$2.5M seed → 18 months → \$1.5M ARR.

USE OF FUNDS (\$2.5M)

42%

Engineering & AI

3 engineers + 1 ML/eval · PMS integrations · carrier playbook tooling

\$1,050K

25%

GTM

Founder-led + 1 BDR + 1 CSM · AAE booth · study club sponsorships

\$625K

8%

Compliance

HIPAA audit · SOC 2 Type 1 · BAA legal · cyber insurance

\$200K

16%

Working capital

DSO contract bridge (long AR) · 6mo cash buffer

\$400K

9%

Founder + ops

Below-market through M18

\$225K

18-MONTH MILESTONES

✓ 65 paying practices

\$1.5M ARR run-rate

✓ 86% gross margin

Validated at scale

✓ 2 DSO LOIs signed

Heartland-tier conversations

✓ 8,000+ resolved claims

Carrier intelligence dataset across 25+ carriers

✓ Series A target

\$8-12M at \$40-60M post on \$4M+ ARR (Q3'28)

 End of deck

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